

RIVERSIDE MEDICAL CENTER

Hospital Discharge Summary

Patient	Elaine Carter
DOB	March 8, 1959 (Age 67)
MRN	DEMO-042781
Encounter	August 13-14, 2026

Reason for admission

Severely elevated home blood pressure with headache. Patient reported a home reading of 178/96 mmHg and presented for evaluation. No chest pain, dyspnea, focal weakness, speech change, or loss of consciousness.

Principal diagnosis

Hypertension, uncontrolled - improved during observation without evidence of acute end-organ injury.

Secondary considerations

- Mild headache, resolved
- Hyperlipidemia
- Osteoarthritis of both knees

Discharge condition

Stable. Ambulating independently. Tolerating diet. Headache resolved. Discharge blood pressure 146/84 mmHg.

Hospital Course

Emergency department and observation course

Elaine Carter presented on August 13 after a home blood pressure reading of 178/96 mmHg with a dull retro-orbital headache. Initial facility blood pressure was 181/94 mmHg. Neurologic and cardiopulmonary examinations were nonfocal. Blood pressure improved gradually with rest and continuation of her home losartan. No intravenous antihypertensive therapy was required.

Diagnostic evaluation

- 12-lead ECG: normal sinus rhythm at 72 bpm; no acute ischemic ST-T changes.
- Basic metabolic panel: creatinine 0.82 mg/dL, potassium 4.2 mmol/L, sodium 139 mmol/L; no acute kidney injury or electrolyte abnormality.
- High-sensitivity troponin: negative on serial testing.
- Urinalysis: no protein or blood.

Medication decision

Because home and observed readings remained above goal, amlodipine 5 mg by mouth once daily was added to losartan 50 mg daily. First amlodipine dose was administered the morning of August 14 and was tolerated prior to discharge.

Discharge Medication Reconciliation

Medication	Directions	Status	Reason
Amlodipine 5 mg	Take 1 tablet each morning	NEW	Blood pressure
Losartan 50 mg	Take 1 tablet each evening	CONTINUE	Blood pressure
Atorvastatin 20 mg	Take 1 tablet nightly	CONTINUE	Cholesterol
Acetaminophen 500 mg	1-2 tablets every 8 hours as needed	CONTINUE	Pain; max 3,000 mg/day
Ibuprofen 200 mg	Do not take unless clinician approves	HOLD	May raise blood pressure

Allergies

No known drug allergies reported.

Medication counseling

- Rise slowly from bed or a chair, particularly during the first week after the medication change.
- Record blood pressure once each morning before medication and again if dizzy or lightheaded.
- Contact the prescribing office for persistent dizziness, fainting, or new bothersome leg swelling.
- Do not stop or change blood pressure medication without clinician guidance.

Results Reviewed at Discharge

Test	Result	Reference / interpretation
Sodium	139 mmol/L	136-145
Potassium	4.2 mmol/L	3.5-5.1
CO2	25 mmol/L	22-29
BUN	17 mg/dL	7-20
Creatinine	0.82 mg/dL	0.60-1.10
Glucose	104 mg/dL	70-110, nonfasting
Calcium	9.3 mg/dL	8.6-10.2
eGFR	79 mL/min/1.73 m ²	Greater than 60
hs-Troponin I	4 then 4 ng/L	No significant delta

ECG interpretation

Normal sinus rhythm, ventricular rate 72 bpm, PR 166 ms, QRS 88 ms, QT/QTc 398/435 ms. Normal axis. No acute ischemic change. No prior tracing available for comparison.

Discharge Instructions

Home monitoring

- Use the same upper-arm cuff and rest seated for five minutes before measuring.
- Keep both feet on the floor, support the arm at heart level, and avoid talking during the reading.
- Bring the blood pressure log and medication bottles to follow-up.

Activity and diet

- Resume activity as tolerated. Stand slowly if lightheaded.
- Follow a lower-sodium eating pattern. Avoid adding salt at the table.
- Maintain normal hydration unless another clinician has advised fluid restriction.

Seek urgent evaluation for

- Chest pressure, severe shortness of breath, fainting, new one-sided weakness, facial droop, trouble speaking, or a sudden severe headache.
- Repeated blood pressure at or above 180/120 mmHg after five minutes of quiet rest, particularly with symptoms.

Follow-up

Appointment	Lakeview Family Practice - August 21, 2026 at 10:30 AM
Purpose	Blood pressure and medication tolerance review
Bring	Home readings, medication bottles, symptom notes, discharge documents

Clinician Sign-off

Discharge assessment

Uncontrolled hypertension improved during observation. No clinical, laboratory, ECG, or biomarker evidence of hypertensive emergency. Patient is appropriate for outpatient medication titration and close primary-care follow-up.

Pending items

None.

Care team

Discharging clinician	Jordan Lee, MD (fictional)
Primary care	Lakeview Family Practice
Discharge date/time	August 14, 2026 at 2:35 PM

Electronically signed for demonstration purposes only

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